

Reporting Child Sexual Abuse in India

Lessons from Developed Nations

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By

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This work is dedicated to all those children who are sexually abused and who I hope to help some day.

LIST OF ACRONYMS

CAPTA	Child Abuse Prevention and Treatment Act
CL-I	Childline India
CL-UK	Child Line UK
CSA	Child Sexual Abuse
GoI	Government of India
IWM	Indian Women's Movement
JJA	Juvenile Justice Act
NSPCC	The London Society for the Prevention of Cruelty to Children
UK	United Kingdom
USA	United States of America
WCD	Women and Child Development

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Abstract

Child Sexual Abuse in families has recently been acknowledged as a real problem in India, after a denial for centuries. However, not only does CSA qualify as an international problem, its underlying causes are also strikingly similar. Most studies have discovered CSA to be a result of power and patriarchy, which makes it a highly secretive affair between adults and children where. USA and UK, having suffered a long phase of denial, are dealing with the problem for over 3 decades. While USA and UK are still facing the biggest challenge of under-reporting of CSA, India hardly has any measures of addressing the issue. Reporting being recommended as the first step in addressing this issue, I tried to compare India with USA and UK to see if India could learn any lessons on increasing the reporting of CSA in India. The study found that India can draw several insights from practices and studies in the USA and UK to tackle CSA.

1 INTRODUCTION

The problem of sexual abuse of children by people known and trusted by them at home has been long ignored by the Indian society, the Government and the Law. The literature on such abuses is very limited. While the society by and large still denies the practice (Segal, 1992), and the legislation still does not distinguish between rape and sexual abuse (Shannon, 2001), the Government of India has come to acknowledge Child Sexual Abuse (CSA) of children at home as an alarming problem which needs immediate attention. In a recent study by the Department of Women and Child Development (WCD) of India, CSA received an extensive coverage for the first time (WCD, 2007).

“The subject of child sexual abuse is still a taboo in India. There is a conspiracy of silence around the subject and a very large percentage of people feel that this is a largely western problem...Part of the reason of course lies in a traditional conservative family and community structure that does not talk about sex and sexuality at all....This silence encourages the abuser ...Very often children do not even realize that they are being abused. [However,] Some deep seated fear has always moved Indian families to keep their girls and their 'virginity' safe ...shows that there is knowledge of the fact that a girl child is unsafe though nobody talks about it. ...the safety net is generally not extended to boys.” (WCD, 2007)

CSA is any sexual act performed by an adult with or in view of a child to seek sexual gratification; it could be anything from photography in nude to fondling of or by the child to penetration of orifices of the child (Finkelhore, 1984; Virani, 2000; WCD, 2007). It has been established by many national and international studies that the abusers are usually known to the abused children (Singh and Kapur, 2001). They could be fathers, step-fathers, mothers, aunts, grand fathers, uncles, friends, siblings or cousins. They are usually in powerful or influential positions over the child and its well-wisher(s) so that in the rare case the child divulges their ‘secret’, they would be safe. In such a milieu, the abuser is more than certain that s/he would go unquestioned and unpunished. Mostly such abuses do not get reported to anyone, usually due to lack of awareness of the abuse or due to fear of blame and punishment of the victim. When they do get reported, most of the times, the process of seeking justice proves to be more destructive than useful, which further aggravates the misery of the abused child. The after effects associated with CSA suffered by the victim range from severe depression, obsessive-compulsive disorders, confused sexual identity, promiscuity, to prostitution, loss of faith and self-esteem, suicide attempts (Finkelhore, 1984; Virani, 2000; Tulir, 2006; Jayakumar, 2007), pregnancy, abortions

etc. (Gupta and Ailawadi, 2005; Fahlberg and Kershner, 2003). Millions of children internationally have already suffered from such disorders due to sheer negligence of the society.

One of the primary concerns towards solving CSA, as found by most studies, is ‘breaking the silence’ and encourage reporting of abuses (Finkelhor 1984; US justice 2000; Virani, 2000; Kaur, 2001; UNVAC, 2006; WCD, 2007). Effective child-friendly reporting practices have two-fold benefits: it meets the inherent need of the victim to communicate and heal; and creates possibilities of preventing further abuse (Finkelhor 1984; Furniss 1991; Parton and Wattam, 1999; Virani 2000). USA and UK have had considerable success in establishing measures that encourage reporting of CSA cases (Wilson, 2000; NSPCC, 2007). This study would analyse those successful measures of reporting in the USA and UK, and assess their applicability towards India.

The underlying reason behind such an approach is that CSA has been a problem faced by all societies, including the developed ones. The root cause of prevalence of CSA such as patriarchy, power, denial and secrecy are strikingly similar to all societies (Segal, 1998). The West today has developed child-friendly laws, policies and practices to tackle CSA but it must be noted that to have these laws and policies functioning in favour of CSA, it had to undergo much transformation; from denial and rejection to acceptance of CSA not so long ago. Lack of laws against families’ ultimate right over the child, subjugation of women and children, lack of awareness of after-effects of CSA etc. were only some of the problems that faced the West before the transformation started (Smart, 2000; Kaur, 2001). Indian society, unfortunately, is still dealing with the same problems. According to Virani (2000), most of the CSA related studies abroad fit the Indian scenario; however, India still has a long way to go to ensure child protection from CSA. Therefore, this juncture is apt to draw lessons from similar experiences of other parts

of the world. While some studies support the idea of considering transferability from West (Segal, 1992), there are several studies that raise valid doubts over transfer of Western models to the South, mainly due to the differences in social and political problems facing both. I would argue that each problem is unique and deserves a holistic analysis to assess whether transfers are feasible or not, rather than generalising non-transferability.

My endeavour through this study is to explore answers to the research question: How can reporting of CSA be increased in India based on experiences of USA and UK, and what are challenges in achieving the same.

To answer the research questions, I first review the relevant literature around the subject of CSA internationally in Section 2 and highlight some gaps relevant to this study. In the Section 3, I present my arguments and analyse the situation based on the data collected from secondary sources like books, journal articles, PhD thesis, articles in the news and internet. The focus of this study is cases of children sexually abused while staying with their families; therefore, only these cases would be referred to and where possible, data would be presented specific to such cases¹. Finally, I conclude in Section 4 by discussing the inferences drawn from the arguments and suggest the policy implications for India.

¹ Literature on CSA in the Indian context was very limited due to lack of study of the problem as well as unavailability in libraries outside India.

2 LITERATURE REVIEW: Reporting of CSA

In this section, I review the literature on issues and debates around prevalence of CSA, its evolution and root causes, impact of legislation on disclosure, and issues around disclosure. The main idea behind this section is to establish that India and the West are similar enough with respect to CSA to form a basis of comparison. Furthermore, I discuss the value added by this study to the existing literature on CSA and also present a conceptual framework for the study.

2.1 Prevalence of CSA as an International Problem

Several international studies have revealed CSA to be a problem faced by all societies. Even though such studies are not comparable due to methodological issues such as differences in definitions of CSA, sources of information etc., several reliable facts were established. To list a few, it was found that younger the age of sexual experience, the more likely it is to be a result of coercion; adults most trusted by the children are most likely to abuse; men abuse more than women do, even though more women are CSA survivors; CSA cuts across all cultures, religions, race, ethnicities, class as well as gender; children of all age groups are at risk, including infants; girls are more at risk of sexual abuse from families (1.5 to 3 times) than boys; CSA is heavily under-reported (Finkelhor 1986; Virani 2000; Fahlberg and Kershner, 2003; UNVAC, 2006). It is also worth mentioning that most abusers have been sexually abused themselves as children, indicating that a safe childhood may act as a CSA prevention mechanism for future children (Sharma and Gupta, 2004). Many of these studies have found fathers and step-fathers to be the abusers in majority of cases and that the secrecy/silence and absence of evidence have made it

the most complex form of abuse (Finkelhore, 1986; Parton and Wattam, 1999; UNVAC, 2006; Tulir, 2006), contributing to non-disclosures.

"...The damage is particularly severe in the context of sexual abuse; particularly as the stigma and shame surrounding ... usually leaves the child ... in solitude. Loss of confidence and belief in the human beings closest to the child can instil feelings of fear, suspicion, uncertainty, and emotional isolation. He or she may never again feel safe or secure in the company of the parent or family member who perpetrated the violence."

The United Nations Secretary General's Study on Violence against Children (Tulir, 2006)

Studies covering English-speaking and Northern European countries, Spanish speaking countries, Greece, North America (focusing on non-European sub-cultures, Western Europe, Central America), the Middle East, Africa, Asia, and the Pacific have found 7 - 36% of women and 3 - 29% of men reporting sexually abuse as children (Finkelhor, 1984; Fahlberg and Kershner, 2003; UNVAC, 2006). Several others have reported CSA for various countries like Germany (91.2%), Geneva (10.9% boys and 33.8% girls), Japan (15.6% women and 5.7% men), and Palestine (38%) and so on. Prevalence has also been established in the Far East and Africa (Fahlberg and Kershner, 2003). Under-reporting of abuse was a problem in most of them. For USA too, CSA was reported for 16% of girls and 5% of boys for the year 1999-2000 (Wilson, 2000) officially. Wilson's (2000) research shows that very few parents sexually abuse their children (Wilson, 2000), contrary to international findings. Given that under-reporting is high in CSA cases (70% for the US), and that children do not tend to complain against parents (Finkelhore 1988; Fahlberg and Kershner, 2003), it could be argued that these figures need

further investigation. Further, a study based on a probability sample of over 2000 young adults from UK in 2000 found that 6% of them were sexually abused as children (NPSCC, 2000).

2.2 Cultural Contexts around CSA: Comparison between India and the West

CSA has thrived for centuries in all societies before being recognized as a problem for children (Kaur, 2001; Fahlberg and Kershner, 2003). The societal code of patriarchy, female subjugation, taboo, secrecy, lack of knowledge about its after-effects, and lack of policies led the abuse to prevail unquestionably. Researchers namely Ambroise Tardieu (1860), Sander Ferenczi (1920s) and Sigmund Freud (1896) tried to expose CSA in families. They were all quieted or forgotten; and Freud who initially theorized that CSA underlies hysteria in adulthood was compelled to change it to an acceptable theory that blames the victims and favours the abuser (Virani 2000; Fahlberg and Kershner, 2003).

Around 1960, the Western society started responding to CSA gradually. In this context, the Battered Child Syndrome by Henry Kempe (1962) remains one of the most referred publications even today (Virani 2000; Galindo; 2006). It must be noted that 60s and 70s were also witnessed rise of feminism and “sex revolution” (Kaur, 2001; Galindo, 2006) in the West (particularly USA), that may have created an atmosphere of acceptance for Kempe’s work. These movements sought to question the established societal norms and power dynamics that favoured males. Feminism best explains the long silence around CSA, given that most victims are females and most abusers males (Parton, 1991; Kaur 2001). Even though until the 70s, the society was divided on CSA as many continued to deny its prevalence (Fahlberg and Kershner, 2003), the

feminist movement had an influence on inclusion of CSA with a proper definition in CAPTA (Child Abuse Prevention and Treatment Act) USA in 1974. USA formed the basis for dealing with CSA in the UK (Finkelhore, 1988; Parton, 1991), as the UK and rest of Europe experienced similar societal transformations. Smart (2000) points to the grim period of denial that prevailed in the UK and Wales until the 80s where, despite the Punishment of Incest Act 1908, sexual abuse by fathers went undetected by professionals. For example, Venereal Diseases among little girls staying with families were all treated as infections contracted from unhygienic practices. The doctors' diagnosis of CSA could lead to removal of the child from treatment as the fathers had ultimate rights over their children. Even here, the UK feminist movement influenced the Law to ensure safety of girls from men (Smart, 2000). Moreover, the advocacy campaigns which led to the UK's Children's Act 1989 paved way for consulting children and seeking their views, something that was never done before. Further to the above developments, there were instances such as the Cleveland Enquiry in the UK in late 90s which drew massive attention to the problem of CSA in families (Kaur, 2001; Parton, 1991; Virani, 2000). Moreover, new studies on short and long term harmful impacts of CSA on children also facilitated acceptance of CSA as a grave problem (Finkelhore, 1988).

Even today, the US and the UK societies and policy makers are divided into two major schools of thoughts: The "Backlash position" (Finkelhore, 1986; Galindo, 2006) which believes that most CSA reports are fabricated, the children generally lie about the abuse (despite reliable evidence suggesting that children do not lie), and that CSA is over-reported; and the "repressed awareness position" which believes CSA to be under-reported and facing another campaign of suppression, referring to Freud's era. Both the groups have influenced the policies but the former has had an upper hand (Galindo, 2006).

While the Western society is still patriarchal, the attitudes of these countries have changed considerably the family is still the primary unit to raise children but the state has the authority to remove the child from the family if deemed fit for his/her safety (Furniss, 1991; Fahlberg and Kershner, 2003; UNVAC, 2006).

“The CRC requires the State to fully respect and support families. But families can be dangerous...sexual and psychological violence, as well as deliberate neglect – has only begun to be acknowledged and documented. Challenging violence against children is most difficult in the context of the family...” (UNVAC, 2006).

The above mentioned factors have been instrumental in changing legislation and attitude of people towards breaking the silence around CSA in the West to a large extent and have also increased reporting of CSA manifold. On the other side of the world, Indian society seems to be undergoing its own transformation, with changing family norms, emphasis on education for all, gender equality, acceptance of homosexuality etc. (www.ndtv.com, www.bbc.co.uk). However, this transformation is rather limited as majority still exhibits adherence to family honour over individual, denial of CSA, “subjugation of women” etc. (Kaur, 2001; Segal, 1998). But notice that the transition itself is comparable to the West in many ways as the society has begun the move from denial to acceptance.

Although the societies have many similarities with respect to CSA, they also exhibit some differences. For example, the Indian Women’s/feminist movement (IWM) that rose in the 1970s and 1980s had limited success, and it faced a downfall in the anti-colonial environment of India which saw them as promoting Western values. The Indian system preferred that its private sphere (where families existed) to be free from Western ideologies (Segal, 1998; Galindo, 2006).

Similarly, being a geographically and socially diverse country, India needs much stronger political will to tackle CSA holistically.

2.3 Impact of Legislation on Disclosure

Legislations have played an important role in enhancing reporting of CSA (Fahlberg and Kershner, 2003). In the UK and the US, the system of child protection is backed by appropriate laws on CSA that have clear definitions of the abusive acts. In some states in US, professionals are mandated to report any suspected abuse and in others, even the community is mandated to do so (Fahlberg and Kershner, 2003). Not only does the law directly promotes reporting, it also encourages reporting through its provisions for punishment of the abuser. For example, offenders in some states are labelled as sex-offenders and are bound to be on probation for few years even after release (Finkelhor and Ormrod, 1999). Additionally, CSA cases are dealt with much greater sensitivity by providing speedy justice, accepting video recordings of child testimonies to be final (saving them from repeated disclosures), interviewing the abused child in friendly and non-threatening atmospheres etc. (Virani, 2000: 156). Such systems that can be trusted by people generally lead to enhanced reporting.

The situation is quite the opposite in India. Despite Constitutional provisions on child protection and ratification to United Nations Child Rights Convention (UNCRC) in 1992 (which implied incorporating UNCRC guidelines and ensuring Child Rights as priority issue), India does not even have a law that deals with CSA. The only Act that deals with children's issues most comprehensively is the Juvenile Justice Act (JJA) 2000 (Kanth, 2004; Galindo 2006; WCD, 2007). However, it must be noted that the JJA (2000) has limited reference to CSA and is

missing in a comprehensive definition to begin with. Section 23 of the Act is about punishing the “person having the actual charge of, or control over, a juvenile or the child, assaults, abandons, exposes or wilfully neglects him/her, causes or procures him/her to be assaulted, abandoned, exposed or neglected ...child unnecessary mental or physical suffering” (WCD, 2007). The child “in need of care and protection” is one “...iv. who is being or is likely to be grossly abused, tortured or exploited for the purpose of sexual abuse or illegal acts,...”(National Institute of Social Defence, www.vakilno1.com).

In the absence of any clear definition of CSA or even sexual abuse in general, proceedings are made as per the existing laws and Indian Penal Code (IPC) that seem closest to the case in hand. Some of those, as described by Virani (2000), Shanon (2001) and Jayakumar (2007), are IPC 375, 376 (rape, limited to penile-vaginal penetration ONLY (Singh and Kapur, 2001); implies males cannot be raped (Singh and Kapur, 2001) and IPC 377 (considers anal sex as an unnatural punishable offense), IPC 354 (‘outraging modesty of women’, only if modesty can be defined and proved); and IPC 511 (intent to rape). Victims usually bear the brunt by either having to marry the abuser or are rendered unmarriageable and face harsh statements from insensitive defence lawyers and judges (Virani, 2000). Low rates of conviction (Singh and Kapur, 2001), inadequate punishment (Galindo, 2006), and a scenario where people cannot trust the judiciary, reporting abuse would seem like a hopeless step. Shannon (2001) argues that changes in Indian legal system are imminent as debates to repeal the archaic laws have initiated (Shanon, 2001).

Justice in India is largely dependent on the discretion of Judges, as few judges are sensitised enough to favour the victim and deliver justice (for example, see Virani, 2000). In this context, a study by Sakshi, an NGO in India, found mixed responses from 109 judges sampled

from wide range sections: 74% believed women should prioritise preserving families in marital disputes; 49% justified occasional slapping of wives; 55% believed women's moral character to be relevant to sexual abuse; 50% said CSA was a feature of "uneducated, depressed, over-sexed groups suffering from sexual disorders; to name a few (Virani, 2000), implying a need for change in attitude. Change in attitude of lawmakers and generating trust in people would go a long way in encouraging reporting (UNVAC, 2006; Shanon 2001).

2.4 Factors that Affect Disclosure of Abuse

The rate of disclosure found in "retrospectives studies" on adults and current child victims ranged from 30 % to 45%, internationally. Several studies have found that disclosure patterns could be affected by factors like relationship with the perpetrator (low disclosures were found for familial abuses than non-familial ones due to fear of hurting or losing family); severity of abuse (Hershkowitz el al., 2007); use of threat to ensure secrecy by the abuser (delays disclosures, sometimes indefinitely); fear of disbelief or rejection from the listener (parton and wattam, furniss); how the abuse was detected (children would disclose sooner if detection happened through medical or forensic methods) (London, Ceci and Shuman, 2005) etc.

Parton and Wattam (1999) argue that the status of children is "socially constructed" (Parton and Wattam, 1999) as innocent, gullible, weak, and dependent individuals who can neither decide for itself nor can cast an opinion. This puts them in difficult position during disclosures (Parton, 1991; Wattam, 1999). On the contrary, experiences show that children have "evolving capacities" and at each stage of development are capable of communicating if trusted

and guided well (Parton, 1991; Wattam, 1999; Boylan and Wyllie, 1999; Fahlberg and Kershner, 2003).

These varying perspectives around disclosure make it a big challenge to interview abused children with tact, patience, sensitivity and care and understand their reasons for seeking confidentiality. Moreover, disclosures are affected by children's interaction with the interviewers. Furniss (1991) and Parton (1991) criticised the interviewing skills of professionals as being ineffective in facilitating disclosures; But Parton (1991) praised the childlines as useful measures of disclosure as they are free of cost and free of judgments, biases or discriminations and ensures complete confidentiality (Parton and Wattam, 1999; Kanth, 2004).

Many studies in the West have been dedicated to unravel the complexity of disclosure practices and have found that the need to disclose their experience of abuse is universal, but is rarely met. Nonetheless, the existing body of literature around disclosure sheds light on several aspects that apply to the Indian context.

From the literature review it becomes clear that CSA is widespread and so is its under-reporting. To enhance reporting, many Western countries have adopted legal and policy measures to ensure that abuses get reported in a timely manner. India, on the other hand, was found at a nascent stage of acknowledgement of the problem; but the situation is comparable to similar phases experienced by the UK and the USA (Segal, 1992). To the best of my knowledge, there is no work on improving measures of disclosure on CSA in the Indian context, and none on learning from the experience of the UK and USA on the same. In the next section, I compare the Indian scenario with that of the West to explore the extent of transferability of reporting measures to India.

Conceptual Framework

CSA is an unethical practice as children are not capable of giving informed consent to any sexual activity with adults. They lack adequate information to be able to realise what it means to engage in sexual activities (Finkelhore, 1984; Virani, 2000; Kaur, 2001). Therefore, it is a wrongful act, which only benefits the adults who perpetrate it. It thrives in silence (Sharma and Gupta, 2004), rarely gets reported and eventually denies children their right to safe and secured childhood. Action against CSA is possible only if report about its occurrence is made and followed up. Reporting CSA can be encouraged by involvement of multiple sectors and child-friendly laws. India can learn many lessons from similar experiences in the UK and the US to generate reports and prevent further abuse, punish the abuser and save the children, and help the survivors recover.

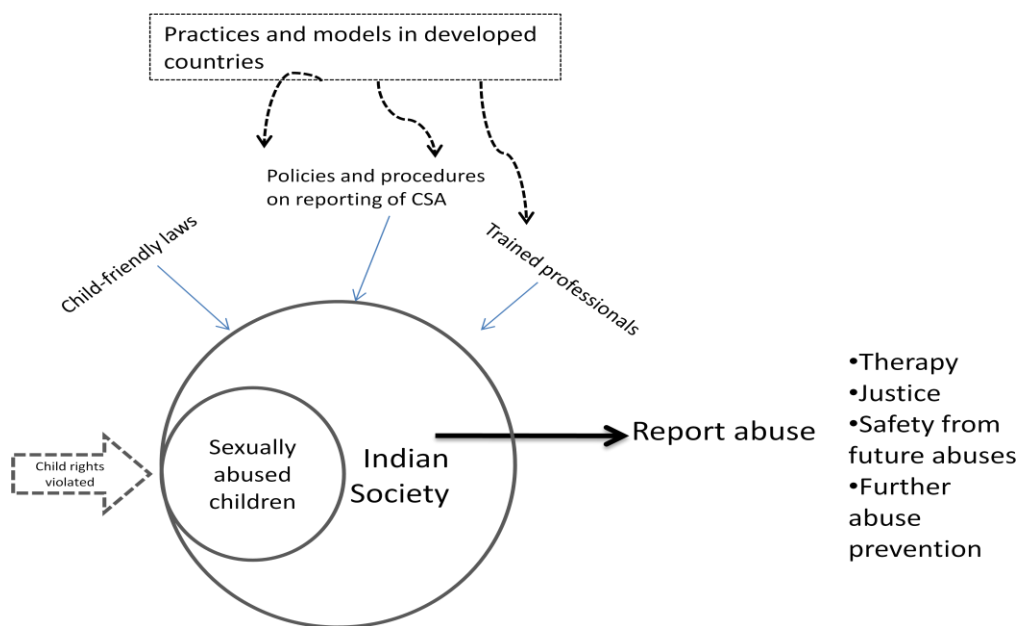


Figure-1 Conceptual Framework

3 WHAT CAN INDIA LEARN: AN ANALYSIS OF MODELS

Having concluded that disclosure measures are important towards prevention and treatment of CSA and that India could learn from experiences of the West, I now compare the models of USA and UK to encourage reporting of CSA and suggest possible lessons for India. Firstly, I present the case of CSA in India to highlight the extent and magnitude of the problem. I follow it by discussing the models of the USA and UK that have been successful in increasing reporting of CSA. Then I would analyse India's position with respect to the Western models, the need for incorporating lessons from those models and what would be needed to incorporate the lessons.

3.1 The Case of India

This section is based on results from two major studies done on CSA in India: the most recent and the only statistically reliable work by WCD (2007) which includes a comprehensive coverage of CSA across India; and Virani (2000), the first study of its kind on an in-depth analysis of the issue, which is more qualitative than quantitative. The respondents of this study were children (5-18 years) from institutions, street and work as well as from home and schools; young adults (18-24 years) and stakeholders from 13 states in India. The study's operational definition of CSA included photography in nude, forcible kissing, exhibitionism, fondling and making children fondle parts, sexual assault (implying penetration of orifices) etc.

WCD (2007) states that:

“India has the world's largest number of sexually abused children, with a child below 16 years raped every 155th minute, and a child below 10 every 13th hour...media and other reports indicate that the unofficial number is much higher”.

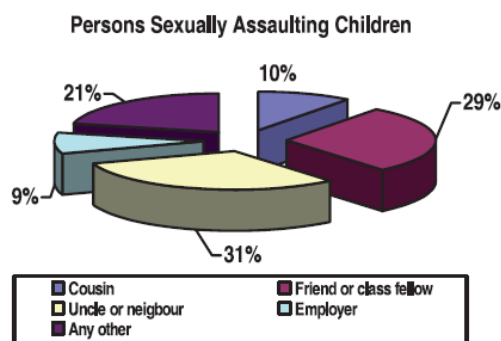


Figure-3 (Source: WCD Report, 2007)

and in-school category, implying families as unsafe for many children). Refer to Figure-2 for break-up of sexually assaulted children among different evidence groups, and Figure-3 for percentages of the assaulters (Note that majority of the assaulters knew the victims). A good example of CSA at home is of an eight-year-old girl who was repeatedly raped by her own father, a police inspector. She escaped from home, ended up in a brothel and was found be suffering from fatal STDs (Virani, 2000).

However, contrary to other studies conducted so far (including Virani, 2000), the WCD (2007) study reports more boys sexually abused than girls. Since data was not segregated by gender for each category of children, it was not possible to estimate the gender gap among the at-home and in-

Overall 53.22% children (53% boys; 47 % girls; 53% at-home and 50% in-school) reported being sexually abused; 22% of them complained experiencing ‘severe forms of sexual abuse’ like fondling, making the child to fondle etc. and 6% experienced sexual assault (of which 13% each were from home

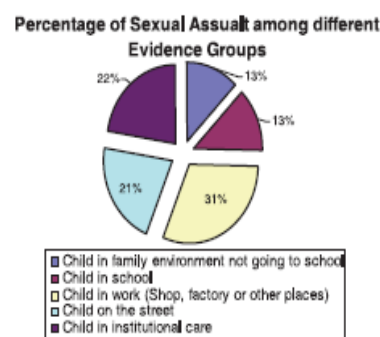


Figure-2 (Source: WCD Report, 2007)

school children. However, Tulir, 2006 had a similar experienced and speculated some plausible explanations. This discrepancy needs further investigation before planning intervention.

A very low disclosure rate was found in this study as more than one-third preferred not disclosing at all, even if they were sexually assaulted. Of those who disclosed, majority disclosed to parents or siblings and only 0.7% to 3.4% to police. Even when children report abuse, they are mostly meted with rejection, blame, or miscommunication (Virani, 2000; Sharma and Gupta, 2004).

Even though the WCD (2007) study has covered CSA extensively, there are several issues that have implications if this study were to be based upon for planning intervention. Firstly, data on fathers, grandfathers and step-fathers as abusers was not collected from children due to sensitivity of the matter. However, responses on these were collected from the youth who indicate fathers (18%) did abuse sexually. All the other national and international studies (Virani, 2000; Fahlberg and Kershner, 2003; Gupta and Ailawadi, 2005, Tulir, 2006, UNVAC, 2006) have found these group of people to form a substantial proportion of abusers. Secondly, women abusers were not mentioned, contrary to other studies (Virani, 2000; Kaur, 2001). Not including women as potential abusers would have implications on intervention plans as they may never get suspected (NPSCC, 2007). Thirdly, there was no mention of disabled children, the most at-risk category. These missing parameters must be investigated before drawing any conclusions from the study for policy changes.

Virani (2000), Gupta and Ailawadi, (2005) and Tulir (2006) have found that CSA cuts across all boundaries in India. To address such a complex problem, participation and coordination of multiple actors from various sectors is needed (Kydd, 2003). Similarly, multiple

sources of reporting abuse must be tapped since anyone could be a potential abuser. For example, when a teacher abuses, parents or health workers or NGOs could report; relying just on teachers could jeopardise the case.

“Comprehensive prevention is impossible without the active involvement of all sectors of society-including health, legal, and social-at all three levels of prevention... This process should involve the child and the child's family, as well as the perpetrator” (Kydd, 2003).

The next subsection is about multi-sectoral approaches adopted by the UK and USA.

3.2 The Primary Child Protection models in the US and the UK

The models for child protection in the USA and the UK involve the professionals and the community and are also backed by appropriate laws to report CSA. Even though under-reporting is high, these measures have had success in reporting of abuses since inception. UN too emphasizes coordination of “Public health, criminal justice, social services, education, human rights organisations, media and businesses” to address the problem of violence against children (UNVAC, 2006).

Child Protection Services (CPS) in the US and NPSCC in the UK are the primary units for protecting children from maltreatments. They are also the first point of contact to investigate abuse. The social workers (SWs) form the frontliners in cases of child abuse and *have the authority to take decisions* in the best interest of the child in matters where the primary caretakers (such as parents) are unable or unwilling to protect their children from maltreatment. The NSPCC is well networked with professionals such as teachers, police, doctors as well as the

community for receiving reports of abuses. They are also required to visit families, especially to assess pre-verbal children (Wilson, 2000; NPSCC, 2007). To cite an example of the procedure, CPS investigates the reports with an aim to strike a balance between keeping the families intact and protecting the child (UNVAC, 2006). They follow well-defined systematic steps while investigating the reports - from assessing urgency of the cases to establishing evidences and substantiating the abuses. They may also approach the juvenile court if the parents refuse cooperation in child removal (Wilson, 2006).

Apart from monitoring by SWs and other professionals, another useful measure for disclosure of CSA is the Childline service. The Childline service allows children to make free calls to share their problems and seek guidance. For example, Childline UK (CL-UK), supported by British Telecom, received 11, 976 calls from children on CSA in 2005-2006 and has counselled 175,000 since inception in 1985 (NPSCC, 2007). According to the report:

“...[CSA] the fourth most common reason for children to call the helpline... Rape was overwhelmingly the most common type of abuse, with 63%... Sexual touching was mentioned by 29%. “

Childlines are not the complete answers to children’s problems but there is a reason why they work. They are easily accessible, reduce delays in disclosure, are free of cost, and ensure confidentiality to the caller (Parton and Wattam, 1999:P149). CL-UK dedicates exclusive sections to CSA to update the extent and magnitude of the problem (NPSCC, 2007). Even though the calls to the childline may not be investigated officially, an analysis of calls does highlight the systemic faults that prevent children from undergoing investigations. For example, children have reported frustration and lack of trust in the legal systems as reasons for not

reporting abuse. Therefore, childline forms an essential part of the infrastructure in dealing with CSA and must not be eliminated from the system (Parton and Wattam, 1999).

3.3 The Child Protection System in India

India does not have a comprehensive model to match the CPS and NPSCC (WCD, 2007; Virani, 2000). The reporting system in India is fragmented with different agencies (the Childline, NGOs, Police, etc.) working separately from each other from collecting reports to following them up, without any common guidelines or convergence at any point. Clearly, there is a need to create CPUs in India (Virani, 2000). For example, the case of a girl whose therapy was terminated by her parents when they found out from her therapist that her own brother was sexually abusing her, which forced her to attempt suicide. This is a case of ‘son’ devouring parents who could not accept their son as the culprit and decided to let their daughter suffer for life (Virani, 2000). Such a case must have been reported to some central unit which had the authority to investigate it. In this section, I analyse India’s position on three parameters with respect to increasing the reporting of CSA cases in a manner that would be multi-sectoral and less fragmented: the infrastructural provisions, the human resource base, and the legal and policy changes needed. The sectors that I refer to are (a) the child protection provisions, (b) the Childline India, (c) the professional sector: medical, education, and the legal sector including police, and SWs and NGO professionals.

3.3.1 The Child Protection Provisions

The closest India's infrastructure gets to the CPU model is through the provisions in the JJA (2000) that mandates presence of Child Welfare Committees (CWC) and Juvenile Justice Boards (JJB) in each district or combination of districts. These groups are comprised of members with qualifications and experience in child development/psychology and experienced social workers (<http://socialwelfare.delhigovt.nic.in/juvenilejustice3.htm>) of whom at least one must be a female. Moreover, the Act also has provisions for child-friendly police to handle cases of children. Even though JWB/CWCs have the authority to decide the protection of the child, the current provisions are only regarding juveniles in conflict with the law and child victims in need of protection and care who are brought before the Board. They are neither the first point of contact in all cases nor are they the primary unit of care and protection. But besides the systemic issues, JWB/CWC does meet some necessary requirements for being a primary CPU from infrastructural point of view.

However, to be effective in handling CSA cases, the JWB/CWC's provisions and services need to be expanded. First, a system for channelling all investigative CSA reports to the CWC/JJB needs to be established. Second, on the legal side, clear and comprehensive definition of CSA needs to be included in the JJA (2000), and the SWs *should be legally authorised to intervene* in cases of CSA. Finally, the existing human resource base may need to be expanded and educated/trained towards handling CSA cases. For example, lessons on determining the urgent cases, art of communicating with sensitivity and firmness with affected children and families, and taking well reasoned decisions in child's best interest; collaborating with other sectors etc. could be learnt from the Western CPUs or CPUs elsewhere, such as Philippines:

CPU (Child Protection Unit) of Philippine has its aim “to prevent further maltreatment and to start the process of healing” of children. “In 2005, the CPU cared for 972 new cases of maltreated children, 81% of whom had been sexually abused”. The cases are handled with in coordination with health, legal and social sector professionals, right from the time of registration. It “ provides legal and police services, judicial hearings, medical services, guidance and support to the child and next of kin, ...therapy or referral to other specialised medical services, when necessary...provides other social services to very poor families, including grants for the child’s school-related costs and interest-free loans for livelihood assistance”. Parents receive training to manage their children and each child in this CPU is assigned a case manager to ensure child safety (UNVAC, 2006).

3.3.2 The Childline India

Childlines have emerged as one of the most popular reporting points for abused children all over the world (Figure-4). India too has an urban-centric childline service which has been quite successful in attending to children in distress. The CHILDLINE of India, founded by Jeroo Bellimoria in 1996, is a 24-hour free phone service that children and adults could call to report or discuss children’s problems.

“Established by the Ministry of Social Justice and Empowerment, Government of India, CHILDLINE India Foundation (CIF) is the central nodal agency for the replication and facilitation of the CHILDLINE service, research, documentation and awareness and advocacy on issues related to child protection” (Childline India, 2007).

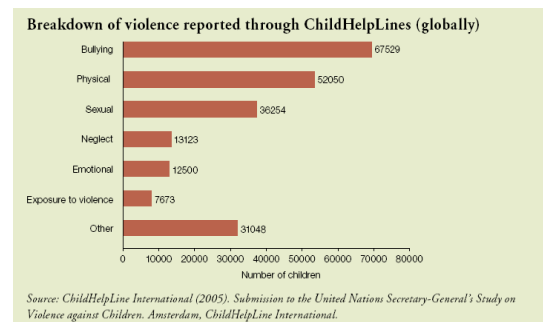


Figure-4 (Source UNVAC, 2006)

Since inception till March 2007, CHILDLINE India (CL-I) had received a total of 11,416,571 calls for different kinds of services for children (Childline India, 2007). Its presence is in 83 cities across 22 states of India. As of 2007 march, about 2 million people call per year compared to only 600-700 calls a months when it as established (Cauli, 2009), clearly indicating its growing popularity among children, especially street and missing children

The CL-I receives calls for problems such as medical help, deaths, tortures, missing children, trafficking, selling of children, child marriages, rape, emotional support, confidence building, general counselling etc. In order to intervene and ensure protection to the child in question, CL-I has successfully networked with several agencies related to children and justice, such as doctors, lawyers, JJB/CWC, NGOs, police, telephone industry etc. and tries to reach children with minimal delay. See Figure-5 (Childline India, 2007).

Even though CL-I intervenes in rape cases, the data in the report does not exhibit sexual abuse (table-1 and Figure-6) (Childline India, 2007). One possible explanation to it could be that may be grouped under silent/confidence building or emotional support category.

Data from 71 cities between April 2006 and Mar 2007

Purpose of call	Number of calls	Percentage
Medical	11372	0.88
Shelter	7712	0.60
Repatriation	7798	0.61
Rescue	3890	0.30
Death Related	189	0.01
Sponsorship	6150	0.48
Missing Children	11123	0.86
Emotional Support & Guidance	434763	33.78
Silent / Confidence Building	353239	27.45
Information / Referral to Services	295077	22.93
Intervention Follow Up	153899	11.96
Unable to Locate Caller	1687	0.13
Total Intervention Calls	1,286,899	100.00

Table-1 (Souce: Childline India, 2007)



Child / Concerned
Adult dials 1098



Connected to a
CHILDLINE
collaborative
agency



CHILDLINE team
rushes to child
within 60 minutes



Child provided
rehabilitation,
Constant follow
up with child

Figure-5, How the Childline India Works (source: Childlien report, 2007)

Except for missing data on CSA, CL-I seems to be growing into a promising point of contact for urban children regarding child abuse. The need for expansion to include the rural areas is already being pursued by CL-I (Childline India, 2007). Since CL-I has proven to be successful in being multi-sectoral, CL-I has the potential to be a central reporting unit connected with the JWB/CWC in such a way that JWB/CWC becomes the first point of contact on reports that need investigation. Calls in need of counselling services could remain confidential with the CL-I. Such an arrangement may take care of the fragmented data recording system in India in which different agencies have duplicate information in separate incomparable formats.

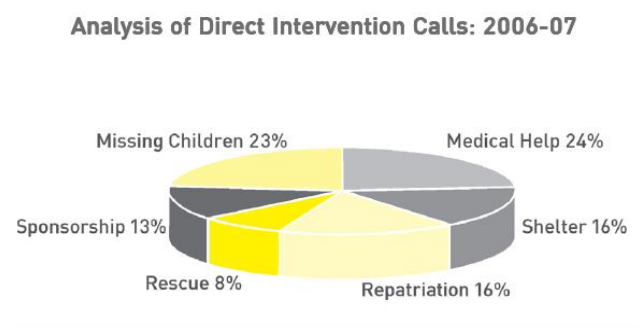


Figure-6 (Source: Childline India, 2007)

3.3.3 The role of Professionals: Medical, Education, Law and Order, Social Workers and NGOs

Most maltreatment cases were identified by schools

Because of the large volume of children attending schools, more maltreated children were identified by schools in 1993 than by all other community agencies and institutions combined:

Schools	54%
Police/sheriff	10
Hospitals	6
Social services	6
Daycare centers	5
Mental health	3
Juvenile probation	2
Public health	2
All others	12

Table-3 (Source: Wilson, 2000)

The CPUs in the West are run by and with the help of several professionals like social workers, police, doctors, health workers, teachers etc. According to 2003 data of the USA (table2), highest number of reports on child abuse and neglect were received from professionals (52%). As well in 1993 (table-3), maximum reports (54%) came from schools alone. This suggests professional to have the potential to report of cases of CSA in India as well, as they are not only available in abundance but also

accessible by most people including children. In what follows, first I would discuss the existing infrastructural base of professionals in India and then conclude with the steps needed to increasing reports from professionals in India.

3.3.3.1 Medical and Health Professionals

The health professionals are spread across India, including the rural parts through the Primary Health Care (PHC) units that are situated for every 25,000 of India's population. The model allows for contact of most women and

Professionals were the most common source of reports of abuse and neglect in 1996

Source of referral	Percent of total
Professionals	52%
Educators	16
Social service	12
Law enforcement	13
Medical	11
Family and community	25%
Friends/neighbors	9
Relatives—not parents	10
Parents	6
Other sources	23%
Anonymous	12
Victims	1
Other*	10

*Includes childcare providers, perpetrators, and sources not otherwise identified.

Source: Authors' adaptation of U.S. Department of Health and Human Services' *Child maltreatment 1996: Reports from the States to the National Child Abuse and Neglect Data System*.

Table-2 (Source: Wilson, 2000)

children with the PHC for issues such as immunization of children, pre-natal and post-natal check-ups for women, promotion of nutrition and basic sanitation, and other diseases as well as referrals to secondary health centres. Even though the services are very poor in some places, there is an established infrastructure which is accessible to people which could be expanded to provide new provisions, such as CSA detection and reporting. An example of such an expansion in the PHC model was during the fifth five year plan which introduced many new activities for family planning (Patel, 2005).

“ Health professionals have an important role ...,except in very remote rural areas, infants and small children are usually taken to the health centre on a routine basis...Given the pressure on health-clinic staff, they need training and capacity-building, as well as improved facilities” (UNVAC, 2006).

3.3.3.2 Teachers

Schools in India are accessible to most children in India, including little children who could go to play schools situated for every 1000 children in rural India. A national sample survey shows an average of only 5% children out of school (ASER, 2008). The issues of poor quality of education and retention are being dealt with aggressively by government and NGOs (www.pratham.org), which is further expected to improve access to schools. The education sector also has an established infrastructure in both rural and urban India. However, the teachers need to be trained and educated on detection and reporting of CSA.

3.3.3.3 Police, Lawyers and Judges

The role of Police, lawyers and judges is probably the most important in the reporting process. Most villages in India have a police station and each district has at least one police station and a court. But there are some major issues with this system. *Sumitra (in Galindo,)*

states that people do not have faith in the police: “They know how the police take the complaints; they do not take it very seriously... A lot of them hate the police since they are heavily influenced by money....” (Galindo, 2006). The WCD (2007) findings indicate extremely low reporting to police by children, ranging from 0.7% to 3.4%, a stark contrast to 30% cases in the US (Finkelhor and Ormrod, 1999). The police and lawyers not only give in to influential parties, they themselves are perpetrators of CSA sometimes. Moreover, the system of interrogation is so rough that it leaves the child ‘re-victimised’ (Kaur, 2001) and in psychological despair (Virani 2000; Singh and Kapur, 2001). Additionally, the judges are themselves driven by the lacunas of laws and their own discretion (as mentioned in literature review) and the verdict is dependent on how sensitive the judge is towards sexually abused children. Thus, such a vast infrastructure is unable to deliver justice due to lack of ethics, power drive, and corruption. UNVAC, (2006) lays emphasis on police being an important change maker in CSA cases instead of being their prime abusers.

3.3.3.4 Social Workers and NGO Professionals

Finally, SWs in India are not considered as professionals at par with other professionals. They are restricted from intervening into the family sphere and they lack the authority to question the parents and remove the child unless gross violation of law happens (Galindo, 2006). Besides the SWs in government settings, a large number of professionals (including those with degrees in social work) are employed by the NGOs. But unfortunately, only a handful NGOs intervene into matters concerning CSA at home. Since this lack of engagement in CSA of SWs and NGOs result from legal lacunas and ambiguous guidelines, changes in legal provisions and policy guidelines are needed to strengthen this sector.

3.3.4 Possible Lessons from the West

Given the above description of existing infrastructure comprising of thousands of professionals in multiple sectors, India has the potential to generate reports from professionals provided some important changes are made to the human resource base and policies. The changes would mainly revolve around education, training and sensitising professionals about CSA and encouraging them to report any suspected cases to CL-I; in this regard, many lessons could be learnt from the West. For example, first, all professional should learn to critically examine parents' versions in suspected cases as children tend to keep 'secrets' of their perpetrators despite the pain, and parents tend to conceal sexual abuses. A doctor in the UK recommended "Don't assume for a minute that children have told you everything..." after his eight-year old sexually abused patient revealed that her father was sexually abusing her after 18 months of counselling (Virani, 2000). Second, techniques used in the West of using toys and 'anatomically correct dolls' to facilitate disclosures by children should be taught to some senior professionals/therapists in India (Virani 2000, Furniss, 1991). Thirdly, all professionals must develop a sound understanding of over-all development of children and the after effects of CSA and its immediate symptoms. For professionals to be effective in communication, they may first need to deal with any issues that they may have with a sensitive subject like CSA before communicating with children in detail. Furniss (1991) and Parton (1991) pointed out that miscommunication is common among well meaning professionals, which prevents children from opening up. There is a vast body of research in the West waiting to be tapped into that lists problems and recommendations towards effective communication (Furniss, 1991, Parton, 1991;

Virani, 2000). Finally, teachers and SWs should organise family meetings to spread awareness of CSA and enable them to accept and handle the problem (Furniss, 1991).

Having talked about drawing insights from the Western systems and procedures, it is also important to consider that CPUs of the West are not free from problems. Their professionals have been criticised of carelessness, negligence and lacking in adequate communication skills by many researchers (Furnis, 1991; Parton, 1991; Parton and Wattam, 1999). Furniss (1991) pointed out that many a times SWs communicate to children that makes them more guilty, feel rejected and leads to partial or no disclosure. Moreover, recent cases of death of small children in the UK have raised several questions on the credibility of SWs role. One such case was the famous case of baby P. who was tortured to death by his mother (www.bbc.co.uk). NPSCC has acknowledged that they were negligent as the social workers were convinced by the “manipulative” mother (www.bbc.co.uk). Further, another problem US is dealing with is that of under-reporting. In the 90s, there was a sharp drop in reporting of CSA cases after a sudden rise in the 80s (Finkelhore and Orkmond, 1999; Jones and Finkelhore, 2001; Almeida et al, 2008). There are several speculative explanations to the issue such as CSA may be declining, or the reports went up because of new recognition but now have fallen to normal level, or the “backlash position” is dominant on public, or changes in policies, or work load on SWs, etc.; but the reports do not suggest any definite cause (Jones and Finkelhore, 2001). Additionally, CL-UK reported children complaining of the procedure of investigation which prevents them from reporting. Some also said that the demand for evidence is too “high” (NSPCC, 2007). The study on the problems not only an indication of a progressive system but also implies ‘learning from mistakes’ for India.

3.4 Conclusion

In this section, I analysed the possibility of improving the system in India on the lines of US and UK to enhance reporting on CSA. From the infrastructural perspective, it turns out that India already has an established and widespread infrastructure that reaches out to a large section of society in order to serve their health, education, and law and order needs. The same infrastructure should be enhanced to incorporate the problem of CSA and increase its reporting. For example, the CL-I already coordinates with the JWB/CWC but to converge CSA reports on investigation into one place, they could be networked in a way that all reports in need of investigation get registered at JWB/CWC. Further, involvement of professionals demand intensive training and awareness, which can be planned and implemented by the government and the NGOs, provided there is a political will to do so.

As India improves its systems and learns from others, there are bound to be many obstacles. The scope of this study was limited to comparison of India with USA and UK; but USA and UK also need insights from other European countries that have better systems, as Parton (1991) rightly points out that no system is ever going to be perfect and therefore the planners and implementers should constantly strive to make things better, without letting complacency creep in. Moreover, it is time India fulfilled its promised ‘conducive environment’ to its children for better lives and brighter futures.

4 DISCUSSION, CONCLUSION AND POLICY IMPLICATIONS

In this Section, I conclude by summarising the major findings from the previous sections and suggest policy implications for India based on the findings.

In the previous sections, I identified that CSA in families by people trusted by children has an international prevalence as is found to be faced by most developed and developing countries; and that CSA remains the most under-reported of crimes perpetrated against children. The most important reason found for under-reporting was the secrecy and taboo associated with the act, which was further a result of power of adults over children and women. Therefore, reporting was found to be the most imperative step to address CSA; and the need of the disclosure was pressing for all children (Sharma and Gupta, 2004). I also identified the similarities India shares with the USA and UK in terms of the culture that fosters CSA, implying that India is lagging behind the USA and UK by a few decades (Kaur, 2001). This observation formed the basis to compare the measures that encourage reporting of CSA in the West with those in India; and it was found that India could incorporate many lessons from the West to increase reporting of CSA. Further, I identified the model for reporting in the USA and UK to be multi-sectoral. These models have several strengths as well as weaknesses (Parton, 1991; Furniss, 1991) but they have enhanced the reporting of CSA since their implementation. Professionals were found to be the most common reporters of CSA in the West. The reason behind the success of these models is the legal provisions that bestow the SWs and police with certain powers over families of children. Based on the model of the West, I explored the infrastructural provisions present in India and identified the changes that may be needed to establish CPUs across India and source reports from the professionals.

Based on the above discussions, I argue that India must consider setting up a CPU in each district with a nodal office at the Centre, on the lines of USA and UK. There are two main reasons for making this argument. First, CPUs are the key agencies needed to tackle cases of child abuse in a manner such that the procedures are uniform all across. This will also prevent fragmented and ad-hoc collection of reports and follow up. Secondly, India has the infrastructure and major provisions in the JJA (2000) to build upon. The provisions need to be enhanced to make the CPUs well-networked and the first point of contact in cases that need investigation. Connected with this is the CL-I that currently performs the role of reporting and intervening through investigation, sometimes with the help of JWB/CWC. The connection between the CPU and the CL-I must be formalised such that the CL-I becomes the entry point for calls regarding CSA but it transfers the calls in need of investigation to the CPU. The network of CL-I is already well established (and expanding); it must be utilised in bringing about this convergence. However, it must be noted that the success of such a convergence lies not only in the policy changes but also in well trained child-sensitive professionals employed in these units who can effectively handle CSA cases. Achieving the target of trained professionals could prove to be challenging because of resource constraints; so it may need to be spread over a reasonably long time or the corporate sector could be tapped into for help.

“Governments should ensure that professionals and non-professionals who work with and around children and their families receive adequate training and ongoing capacity building which includes basic information on children’s rights and the law, violence against children, its prevention, early detection and response, non-violent conflict management, and children’s rights. In addition, workers must have a clear understanding of the physical, sexual, emotional and cognitive development.... (UNVAC, 2006).

To achieve this, while some lessons from the West seem applicable, others may need to be modified to suit the specific culture and milieu of the state. For example, awareness generation and sensitisation methods must be unique to the local context where successful conventional methods such as folk songs, dances and plays may be combined with modern ways of communication such as books published by Tulir (an NGO addressing CSA in India) to ensure effectiveness (Galindo, 2006). In addition to sensitising, awareness on where to report should also be priority as “Retrospective studies” that interviewed young adults about their childhood experiences revealed that the majority of child victims did not talk to anyone or approach child protection services during their childhood, ...not knowing where to go for help...” (UNVAC, 2006).

Another key area to improve reporting on CSA is to sensitise the police and the judiciary, and improve proceedings to ensure child-friendliness. People in India have clearly lost faith in their system of law and order. This is one of the reasons why perpetrators get the confidence of getting away easily after committing the crime. In this context, amendment of the JJA (2000) with a comprehensive definition of CSA and authorisation of intervention into family matters in case of CSA by workers must be emphasised.

“Governments must develop child-sensitive procedures for investigating cases of violence, which avoid subjecting the victim to multiple interviews and examinations. ...The stress of court proceedings can be reduced through the use of ...as video-taped evidence, courtroom screens, and witness-preparation programmes...” (UNVAC, 2006).

In this regard, even though the US and UK systems are developed to be child-friendly, they also need to improve their systems, based on feedback of children to the CL-UK (NSPCC, 2007). Rewarding the reporters, as suggested by UNVAC (2006), could also be experimented to enhance reporting although such option must be closely regulated to ensure fair-use.

Finally, it must be acknowledged that CSA is different from other kind of abuses (such as non sexual physical and emotional abuse) as suggested by many studies (Virani, 2000; Fahlberg and Kershner, 2003; UNVAC, 2006) and thus it needs special modes of interventions. The key issue is that since “sex” is involved people are hesitant to talk about it and thus its detection is a huge challenge. Clubbing CSA policies and guidelines with other kinds of abuses would oversimplify this complex problem and would adversely affect the intended outcomes. It would also affect the therapy of survivors as CSA survivors have much deeper wounds and loss of faith comparatively. In addition, girls are more at-risk than boys in most societies (Fahlberg and Kershner, 2003), especially in India where most girls are entrapped in household chores and are restricted from leaving home, which makes reaching out for help a bigger challenge for them. *“...States [must] ensure that anti-violence policies and programmes are designed and implemented from a gender perspective, taking into account the different risks facing girls and boys in respect of violence (UNVAC, 2006).*

To conclude, the analysis and recommendations made in this study have highlighted several possibilities of changes in India based on the US and UK models. However, while presenting my arguments and recommendations, I am well aware of the fact that the Indian scenario is different from that of the West due to extreme poverty, petty and entrenched corruption, great gender divide and many other unique social and economic problems. But I argue that these problems would present themselves as hurdles in almost every developmental

planning; gender issues and corruption cut across all sectors. While it is a harsh reality that these hurdles would slow the process of change, it is also true that higher level planning needs to be progressive. It is at the stage of operationalisation r a reality check on where India stands and it is not possible to comment whether the initiative would succeed or not due to several unforeseen impediments. On the brighter note, separate efforts are already being made to address these issues which should be able to minimize their impact. Moreover, India currently is in a state of flux as more and more people get vocal about the social problems and media also plays a major role in encouraging such changes. Therefore, as Finkelhore (1988) aptly states:

“This particular moment is a formative one in the development of international child abuse initiatives, and it calls for both urgency and caution, mobilization and reflection. The urgency and mobilization are required by the large dimensions of the problem and the pain and suffering of those involved. The caution and reflection are required by the complexity of the task, and the variety of considerations that need to be taken into account. ...New ideas need to be developed and solicited. ...the ideas most likely to succeed in alleviating these threats to children’s health, development, and survival” (Finkelhore, 1988).

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